جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : مى حسين مصطفى حسين
المحافظة : القاهره **الحي / المدينة :** مدينه نصر **العنوان :** الدينارى
رقم البطاقة : 29107050103062 **رقم التليفون :** 01066231201 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 05-07-1991 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مى حسين مصطفى حسين
National ID: 29107050103062

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





DMS

٢٠٠٢٦١٤١٢٧

* زولا حسن + ثامر كفير

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستافروسي انترناشونال مصر

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

MDT FORM 24/11/2025 G

Name	[illegible]	MRN	٢٠٩١٠٧٠١٠٢٠٦٢
		ID	C.Co/ ٢٢٦٢
Age	٣٤ سنة	Phone	٠١٠٦٦٢٣١٢٠١
Menopausal status	pre	Marital status	Not married
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	Post OP
CO morbidity			

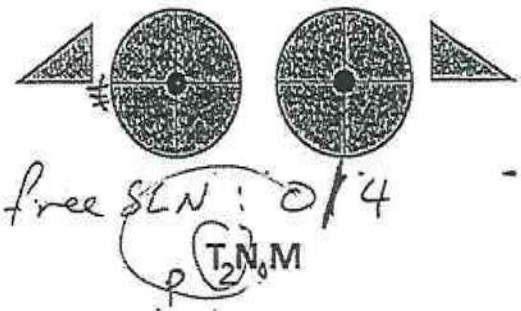
Upfront RA WLE & SLN

Clinical Data

BX: 2.5 x 2 x 1.5 cm IDC GII

ER:	PR:	HER 2:	Ki67:
8/8	7/8	+2 Not ampl.	7%

R - free margins
- DCIS: focal (high grade) L



free SLN: 0/4

T_2N_0M

Sonomammography:

Mets Workup:

C. Co/C.1/Co

Recommendations:

1. Adjuvant Hormonal ~~Hx~~ (Zoladex + TAM).
2. Fertility Counseling (Not Married)

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ [illegible]					

MDT Form
[[DOCTYPE:MDT_FORM]]

USTAVE/
OUSSY
CANCER CAMPUS

Patient Name	: الأستاذه / مى حسين مصطفى حسين	Request No.	: 13003556
Patient No.	: 29107050103062	Request Date	: 20/08/2025
Path No.	: SR-1146/25	Verification Date	: 25/08/2025
Sex	: Female	Financial Category	: جوستافت روسي - نفقة الدولة
Age	: 34 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

REVISED
Clinical Data: Right breast mass at 10 o'clock.
Nature of Specimen: Referred block & slides (Tru-cut biopsy & FNA).

Gross Examination:
One referred block labelled by (656-5-2025). Together with, 10 referred slides including 3 H&E stained smears with one cell block, 2 H&E stained core biopsy & 4 immunostained slides for (ER, PR, Her2neu & Ki67).
Technique:
- Section were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- SISH.

Microscopic Examination:
- Referred smears with cell block (FNA):
Cellular smears entangling groups of malignant epithelial cells showing high nucleo/cytoplasmic ratio, arranged in clusters among eosinophilic background.

- Core biopsy slides:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
* 2% Low grade DCIS (solid pattern).
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* IPT:
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / مى حسين مصطفى حسين	Request No.	: 13003556
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Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).(REPEATED IN OUR LAB)
- SISH: Not amplified.
(Average Her2 copy No.: (1.9), Her2/CEP17 ratio: (1.3)).
- Ki67%: 7%.

Diagnosis:
* Right breast mass, Referred block & slides (Tru-cut biopsy & FNA), Both:
- Invasive duct carcinoma, Grade 2, with 2% low grade DCIS (Solid pattern).
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
(Average Her2 copy No.: (1.9), Her2/CEP17 ratio: (1.3)).
- Ki67%: 7%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or

Verified by :
Dr.Duaa Mohamed

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Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.
ISH.groups:
The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Prof .Dr.Maha Guimei
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

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C A N



ID: 2752859 Name: Mai Hussein Moustafa
Date: 14/06/2025

High Field Dynamic MRI of Both Breasts

Technique :

- **Axial T2 (plate 1).Axial STIR (plate 2).**
- **Sagittal postcontrast 3D TFE of the right breast (plate 3).**
- **Sagittal postcontrast 3D TFE of the left breast (plate 4).**
- **Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5,6),Time intensity curve(plate 7).**

Findings:

- Bilateral scattered mixed densities of glandular condensation .
- Right UOQ microlobulated lesion of Low SI in T2 and bright SI in STIR measures about 20x15mm showed focal enhancement , the kinetic data showed rapid initial rise of the curve reaching a peak of maximum enhancement at 2.30 minutes followed by wash out (Type 3 curves).
- Bilateral scattered foci of punctuate parenchymal enhancement ,the kinetic data showed continuous steady rise with no wash out (Type 1 curves).
- Normal skin thickness and contour.
- No nipple or skin enhancement.
- There are likely non-specific axillary lymphadenopathy.

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S C A N

OPINION :

Right UOQ suspicious lesion categorized as **BIRADS 4c** based on the morphological and kinetic data for **tissue diagnosis .**

Bilateral scattered foci of punctuate parenchymal enhancement categorized as **BIRADS 2** based on the morphological and kinetic data .

Bilateral mild mammary fibroadenosis.

Dr. ENGY ADEL,MD
Assistant Prof. of Radiology. Cairo University

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	

alfa
SCAN

Name: Mai Hussein Moustafa Hussein ID: 4507612
Date: 12/06/2025

^{18}F-FDG PET/CT examination

Clinical Indication:

- *A patient presented with pathologically proven right breast invasive duct carcinoma, for assessment.*

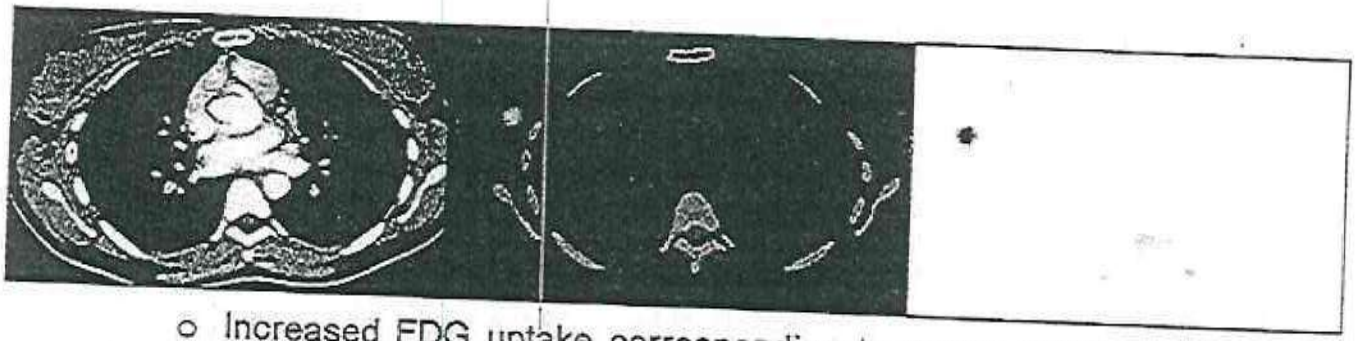
Administered activity: 222 MBq. ^{18}F-FDG **B.G.L.** : 72 mg/dL

Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Diagnostic multislice CT examination after IV contrast administration of the same region.

I. *PET/CT Findings:*

- Right breast:



  - Increased FDG uptake corresponding to upper outer quadrant lesion, SUVmax 6.4 and measures 1.8x1.5 cm.
  - No chest wall or skin invasion.
- Left breast: no FDG avid lesions.
- Lymph nodes:
  - No significant FDG activity within small right axillary nodes reaching 0.9x0.8 cm.
  - No significant FDG activity within small left axillary nodes with preserved fatty hila, *reactive.*

- *Otherwise, no FDG avid or pathologically enlarged lymph nodes.*

- Lungs:
  - Tiny bilateral pulmonary nodules with no significant FDG activity, *for follow-up.*
- Liver:
  - No FDG avid focal lesions.
  - No dilated intrahepatic biliary radicles.
- FDG activity within retrosternal thymic soft tissue.
- Pelvic organs:
  - No FDG avid lesions.
  - FDG activity within endometrium and right adnexa, *likely functional.*



- Bone marrow: no FDG avid lytic or sclerotic lesions.
- Diffuse FDG activity within brown fat in neck, shoulders, and mediastinum and paravertebral regions, ***physiological.***
- Brain: no FDG avid intra-axial lesions.

II. Diagnostic CT Findings:

- No pleural, pericardial or peritoneal collections.
- The spleen, pancreas, adrenals, kidneys, aorta and IVC appear unremarkable.

OPINION:

- Metabolically active right breast neoplasm as described.
- No significant metabolic activity within small bilateral axillary lymph nodes.
- No other hypermetabolic lesions detected.

Radiologist
Dr. Mohamed Hoseny, MD

Nuclear Medicine Physicians
Dr. Sherif El-Refaei, MD, EBNM
Dr. Asmaa Magdy, MD